Which treatment this guide covers National Cancer Control Programme (NCCP) 00565a covers first-line treatment of adults with locally advanced or metastatic non-small-cell lung cancer (NSCLC) with an activating EGFR mutation. This is daily dacomitinib alone; it is not an adjuvant regimen or a chemotherapy combination.

NCCP 00565 · VERSION 3

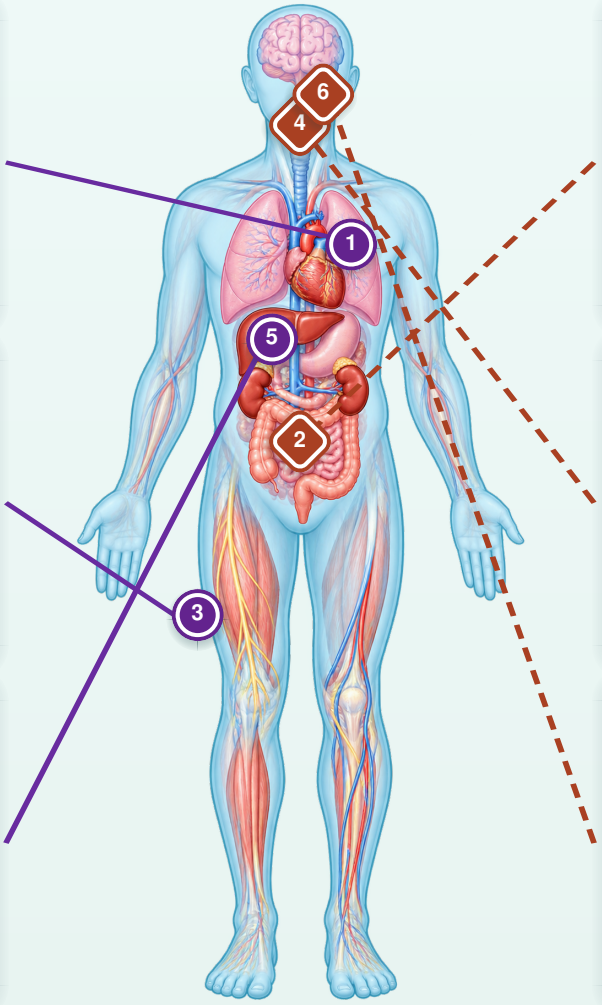
Dacomitinib monotherapy

The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

dacomitinib

Possible lung inflammation needs urgent assessment.

2 Diarrhoea and dehydration

dacomitinib

Diarrhoea can become serious quickly and may affect the kidneys.

3 Rash, nail changes or peeling

dacomitinib

Early treatment can help; severe reactions need urgent review.

4 Mouth soreness

dacomitinib

Report difficulty eating or drinking.

5 Yellow eyes or skin

dacomitinib

Liver injury may need blood tests and treatment review.

6 Eye pain or blurred vision

dacomitinib

Eye inflammation or keratitis needs assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or peeling skin reactions; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

Know the next step.

Taking treatment

The usual starting dose is 45 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription; some patients start lower because of liver function. Treatment continues while it helps and side effects remain acceptable.

Take at approximately the same time each day with or without food and swallow with water. If you miss a dose or vomit, do not take a replacement or extra dose. Take your next prescribed dose at the usual time the following day.

Checks and follow-up

Confirm the activating EGFR test, first-line treatment setting and prescription. Check blood count, kidneys, liver and bone profile, and arrange baseline staging scans. Review lung symptoms, bowel and skin health, all medicines and pregnancy precautions. NCCP requires regular blood count, kidney, liver and bone tests but does not specify a fixed interval; your team sets appointments. Diarrhoea often starts early, so keep the symptom plan ready from the first dose. Scans follow your Consultant's plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Diarrhoea and fluids

Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.

Skin, nails and mouth

Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.

Nausea medicines

Dacomitinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.

Medicine interactions

Acid-suppressing medicines can reduce dacomitinib exposure. Proton pump inhibitors should be avoided if possible; ask pharmacy about alternatives. NCCP and current product information give different timing before H2 blockers, so obtain a written individual schedule. CYP2D6 medicines also need pharmacy review. Do not change medicines yourself.

Pregnancy, fertility and breastfeeding

Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 17 days after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.



Your complete treatment reference

https://sactcheck.com/patient/00565/

Public information only. No personal information is encoded.